

8th July

Post 1- 8th July

Indian philanthropy is in the middle of a remarkable decade. Giving crossed USD 16 billion in FY2024 and has been growing around 12% a year, led by a new generation of UHNI and HNI donors (Indian Philanthropy Report, Bain & Dasra 2025).

As this generation matures, more of them want to understand what their giving actually changes – and to put evidence and data behind those decisions.

That appetite is real. What's largely missing is the infrastructure to meet it.

Around the world, a handful of the best-known evidence-led advisors have helped move billions of dollars, and helped donors identify the interventions that reach hundreds of millions of lives rather than thousands. None of them work in India, which leaves donors here with few places to turn for that kind of rigour. In a country where roughly 160 million people still live in multidimensional poverty (NITI Aayog, 2022–23), and one of the fastest-growing philanthropy markets in the world, that is a real gap.

We think it is also one of the most tractable ones in Indian development. Donors new to giving are genuinely open to guidance, and guidance shapes where capital lands. Our first agriculture engagement helped move a first-generation donor to a confident USD 1M commitment toward evidence-backed organisations.

That is the work Raising Impact exists to do: advisory that leads with evidence and data, so serious philanthropic capital flows to what actually works – and every rupee reaches as many lives as it can.

We're a small team building this in the open, and we're hiring two researchers to do it with us. Details here: <https://raisingimpact.org/careers/>

A question worth sitting with: what would Indian giving look like if evidence were where we started?

#Philanthropy #EvidenceBasedGiving #IndiaDevelopment #EffectiveGiving #EffectiveAltruism

India gives more than ever before. It just doesn't yet give on evidence.

Indian philanthropy crossed USD 16 billion in FY2024, growing ~12% a year, driven by a new generation of UHNI and HNI donors who think analytically and want data-driven ways to give (Indian Philanthropy Report, Bain & Dasra 2025).

But here's the gap that made us start Raising Impact.

Despite ~160 million people still living in multidimensional poverty (NITI Aayog, 2022–23), and one of the fastest-growing philanthropy markets in the world, India is served by none of the major evidence-led players. @GiveWell, @Founders Pledge, @Coefficient Giving have moved billions globally, but they don't operate here. Rigorous, evidence-based advice for donors with philanthropic capacity of USD 1 millions is effectively absent.

We think that's not just a gap. It's one of the most tractable ones in Indian development.

Donors new to giving are open to guidance, and guidance changes where capital lands. Our first agriculture engagement moved a first-generation donor to a confident USD 1M commitment toward evidence-backed organisations.

That's why RIO exists: to help serious philanthropic capital in India flow to what actually works to maximize the impact of per rupee spent.

We're a small team doing this in the open, and we're hiring two researchers to do it with us. More details here: <https://raisingimpact.org/careers/>

What would change in Indian giving if evidence were the default, not the exception?

#Philanthropy #EvidenceBasedGiving #IndiaDevelopment #EffectiveGiving #EffectiveAltruism

Word Count: 238

23rd July

At RIO, early childhood care and education was our first research pilot. We chose it for four reasons: the sector already had enough maturity to build on, it is an enduring development problem, the funding pathways already exist for it, and it holds real salience with government priorities.

Kangaroo Mother Care, skin to skin contact for underweight newborns, is one of the most evidence-backed, low-cost interventions in newborn survival, recommended by WHO and linked to significant reductions in mortality and infection. Early in this work, while building a cost-effectiveness case for it, we ran into a specific problem on the evidence side.

We wanted a number that tells us how much KMC actually reduces a newborn's risk of dying, one grounded in evidence that applies to Maharashtra specifically. The best available review looks at 21 trials on KMC, 8 of them conducted in India, which sounded exactly like what we needed. But the headline result you'll see quoted everywhere, a 40% reduction in mortality, isn't calculated from all 21 trials. It comes from combining a smaller subset of those trials into one average number. At least one Indian trial appears to be part of that smaller group, but it's pooled together with trials from several other countries, so the number reflects an average across very different health systems, not India specifically. On top of that, in several of the underlying trials, including some of the Indian ones, it's unclear whether mothers in the comparison group got the same breastfeeding support and encouragement to be involved in their newborn's care as mothers in the KMC group did. If that differed, part of what's being measured as KMC's effect may actually be these other practices, not skin to skin contact alone. So 40% likely overstates what a real Maharashtra ward, at real-world compliance, would get from KMC specifically.

So we didn't use 40% as our answer. We used it as a ceiling, then discounted it three ways: how much skin to skin contact already happens informally in Indian wards, how many babies would actually receive it once a program scales past a trial setting, and how much of the mortality benefit is attributable to KMC alone versus the bundled care it typically travels with. Multiplying those three discounts brought our working estimate down to about 14%.

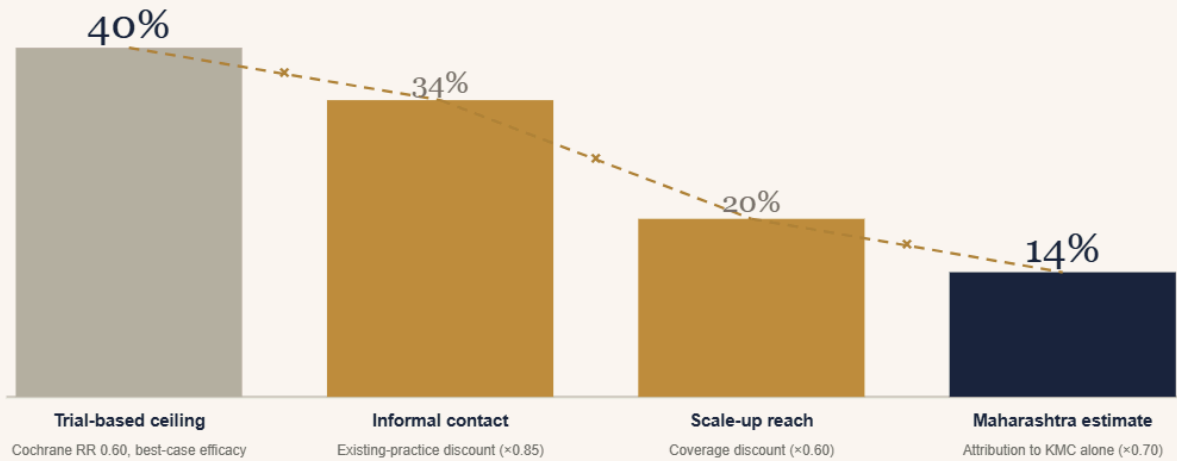
The learning: a global effect size is a starting point, not an answer. The real work is naming what's different about your context, one assumption at a time.

#Philanthropy #EvidenceBasedGiving #IndiaDevelopment #EffectiveGiving #EffectiveAltruism



From trial ceiling to a real-world estimate

Each step applies the actual discount from RIO's KMC model; a multiplicative scale-down, not a subtraction - to move from best-case trial efficacy to the expected impact in a Maharashtra facility.



Multiplicative decomposition from RIO's KMC cost-effectiveness model — $40\% \times 0.85 \times 0.60 \times 0.70 \approx 14\%$.

6th Aug

Hook:

We spent months looking for the best way to raise a smallholder farmer's income, and where philanthropic capital has the most leverage.

Ambitious Impact recently published its house view on increasing incomes in low- and middle-income countries. Its argument: raising incomes is one of the most direct ways to improve well-being and agency, and the interventions worth funding are those that move income per dollar spent, held to a cost-effectiveness bar of ~\$18 per income doubling. Agriculture is named among some of the promising paths. It's worth reading in full, Link:

<https://ambitious-impact.gitbook.io/ambitious-impact-docs/explainers-and-house-views/live-house-views/increasing-incomes-and-supporting-growth-in-low-and-middle-income-countries-lmics>

It's close to the question that drove our own work: for a small or marginal farmer in India, where can philanthropic capital move income most effectively, and which organisations can deliver it?

What we learned:

- Raising the price a farmer gets for an existing crop matters, and much of the sector already works on it. But the income ceiling is often set by what a farmer grows, not only what they're paid for it. Diversification into high-value agriculture, horticulture and allied sectors is a well-recognised pathway, and we assessed it as where philanthropic capital has real leverage
- The headroom is large, but the barriers of knowledge, finance, inputs, market access, and price volatility keep smallholders from crossing into it on their own. Any intervention that ignores these doesn't scale.
- Delivery matters as much as design. Small and marginal farmers moving into diversification need hands-on, sustained support, the kind of high-touch handholding that product-led business models aren't built for at this segment yet. That points toward credible mid-size NGOs as the right delivery route, working alongside government schemes.

We took this from a landscape scan through expert interviews to a structured grant call for organisations already showing results with evidence in India's highest-need states.

One note of association we're glad to share: Our founding researcher, Pradnya Nikam, trained through AIM's cost-effectiveness Research Program, the same rigour that shapes this house view. It's the tradition we're building from, adapted for the Indian context.

Stay tuned for more information on our research work and cost effectiveness analysis for 'Farmer Income increase'

August: Series post

POST 1: Introduction + Cause-Area Selection

Hook Line:

We don't choose cause areas by just asking where the need was greatest, we look through multiple filters

Over the next few posts, we're opening up how Raising Impact actually works: how we choose a cause area, interrogate it, validate it with experts, and pressure-test the evidence, before any money moves. This is the method that sits underneath every recommendation we make.

It starts with a question most giving skips: **which problem should we even be working on?**

We looked at six cause areas. We began with two: Early Childhood Care and Education, and Income Enhancement for farmers.

Here's what might surprise you. **We didn't pick purely on need. Need is necessary, not sufficient.** A problem can be severe and still be a poor use of philanthropic capital if the sector isn't adequately ready to absorb it, if no proven solutions exist, or if there's no one credible to deliver.

So we scored each option on four things: how mature the sector is, how much funding already flows to it, how enduring the problem is, and how well it aligns with government priority.

Mental Health, Air Quality, Water and Food Systems, and One Health made the shortlist. ECCE and Livelihoods made the cut, because they scored where it mattered: enduring problems, real funding pathways, strong policy salience, and enough sector maturity to build on.

Stay tuned to learn about - Once a cause area is chosen, how do we go deep? We'll break down the framework we use to interrogate.

Priority Cause Areas		
Sector	Education & Health	Livelihoods
Theme	Early Childhood Care & Education (ECCE)	Income Enhancement
Sector Maturity Issues are well-understood, with established metrics and long-standing stakeholders driving action	Mainstream	Emerging
Existing Funding Volume and diversity of philanthropic and institutional capital	Low - Medium	High
Age of Topic How long the theme has had sectoral focus and investment	Enduring	Enduring
Prediction of Salience Priority level for the present government	Somewhat Strong	Strong

Other cause areas considered: Mental Health (Health), Air Quality (Climate), Water & Food Systems (Climate), & One Health (Climate & Health)

POST 2: Cause-Area Research (NLIO)

Hook Line:

India's richest state is one of its hardest places to be a young child.

Once we chose early childhood as our pilot cause area, we needed a way to interrogate it, not just confirm that it matters, but find where philanthropy could actually shift the needle. We use four questions. We call it the NLIO lens: Need, Leverage, Interventions, Operators.

In this post we will cover Need and Leverage: the two you can answer from data before speaking to anyone.

Need asks where the problem is worst. What came as a shock to us is that Maharashtra has the highest concentration of CSR funders in the country, yet underperforms on 11 of 17 child well-being indicators such as stunting, wasting, infant mortality, and pre-school attendance. (need to add source)

Leverage asks where philanthropy can add value the state can't. Government spends around USD 1.8 billion a year on early childhood in Maharashtra. Philanthropy adds about USD 6.1 million. That is roughly 0.3% of the picture. (need to update)

That ratio reframes the job. At 0.3%, philanthropy's role can't be volume. It has to be leverage: funding what the system won't, and proving what the system could scale.

Need and Leverage tell us where to work and why. They don't yet tell us what works, or who can deliver it. That's the other half of the lens.

Next: our secondary data flagged some surprising places as underserved. We took those findings to people who work on the ground, and what they told us changed the map.

Guiding question: What are the optimal pathways for deploying philanthropic capital to maximize impact?



1 Where is the greatest need?

- How is India performing globally?
- How are states faring within India?
- Are there regional nuances?



2 Where is the highest leverage?

- How much is the Indian state spending?
- How much is spent as philanthropy?



3 What are the proven interventions?

- What are proven practices/interventions?
- What kind of evidence is available?
- What are the cost effectiveness of these interventions?



4 What and where are the reliable operators?

- Which are some credible operators?
- What is ecosystem wide infrastructure?
- What types of partnerships are required?

POST 3: Interventions + Operators (This post do not have image)

Our data said Thane, Pune, and Kolhapur were failing their children. Experts told us why, and then told us what to do about it.

These are among Maharashtra's most prosperous districts, with strong government and philanthropic spending. Yet our secondary analysis flagged them as underperforming on child health. On paper, that looked like an error in our data.

So we did what we always do before trusting a desk finding: we took it to people who work on the ground. They confirmed it, and explained what the numbers hid. Prosperous districts hold large urban slum populations and fast-growing outskirts, and those pockets are often the least served. District averages bury them.

But validation is only half of what those conversations do. They also tell us which kinds of solutions actually work.

Early childhood interventions fall into four types. Awareness, like counselling on feeding and newborn care. Capacity building, strengthening the skills of frontline workers. Systems strengthening, improving how existing services are delivered. And direct beneficiary transfer, giving resources straight to families.

Here's the part that should reshape how we fund. Around 60% of philanthropic funding for early childhood in India goes to infrastructure and direct delivery. Yet the evidence points to awareness and systems interventions as far more cost-effective per outcome, often by an order of magnitude.

The most-funded solution is rarely the most effective one.

And knowing which solutions work still leaves the last question of the lens: who can actually deliver them. A proven intervention is only as good as the organisation running it, so we map the credible operators on the ground, the ones with the reach and the track record to make it real.

Need, Leverage, Innovations, Operators. Four questions, and only now do we have the full picture of where philanthropy can move the needle.

Next: A promising intervention backed by a capable operator still isn't a decision. Before we recommend it, the evidence for intervention itself has to clear a bar, and we'll show you how we set it.

POST 4: The Evidence Bar (Evidence Taxonomy)

The best evidence for what works in India often isn't Indian.

Once experts help us see which solutions are promising, we still have to ask the harder question: how good is the evidence that they actually work? A promising intervention isn't a decision. It has to clear a bar.

We rank evidence by how much weight it can bear. Meta-analyses and systematic reviews at the top, then randomised controlled trials, then quasi-experimental studies, cohort studies, pre-post studies, and expert consensus. The higher up, the more confidently we can act on it.

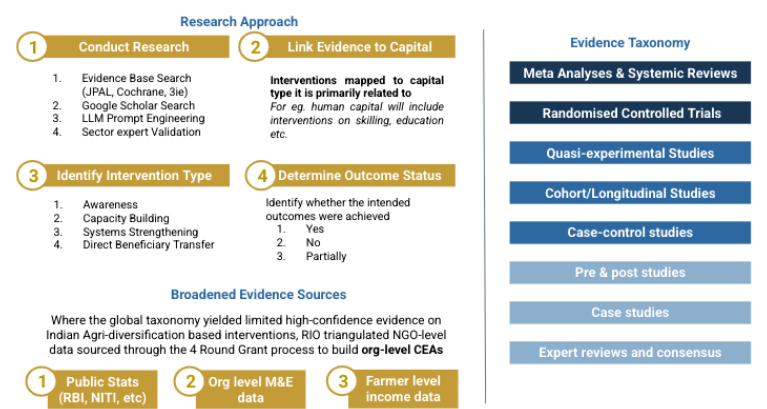
The problem: for many interventions in India, the top of that ladder is empty. The rigorous trials were run elsewhere.

We don't treat that as a dead end, and we don't lower the bar. Instead, we broaden the base. Where Indian trial evidence is thin, we triangulate: strong global evidence, public data from bodies like NITI Aayog and RBI, on-the-ground monitoring data from implementing organisations, etc.

The discipline is holding two things at once. Take the best available evidence seriously, wherever it comes from. And stay honest that evidence from one context rarely transfers cleanly to another. That second half is the hard part: taking a global effect size and asking honestly what it means for one ward in Maharashtra. It's slow, unglamorous work, and it's the difference between evidence that sounds good and evidence you can fund on. A detailed explanation on we build in this discounting is covered in our KMC example here: [Link to KMC post](#)

That's the method, from choosing a cause area to holding the evidence bar. We'll go deeper on how we turn this evidence into a cost-effectiveness number in a longer piece soon.

For now, a question worth sitting with: if evidence this demanding is what it takes, how much of today's giving would clear the bar?



Blogs

Blog 1:

Why evidence-led giving has no home in India (yet)

A generation of analytical donors is reshaping Indian giving. The evidence-led guidance they're looking for hasn't caught up. That's why we started Raising Impact.

India is in the middle of an unusual decade for private giving.

Domestic philanthropy crossed USD 16 billion in FY2024 and has grown at roughly 12% a year, driven largely by a new cohort of wealthy donors (Bain & Dasra, 2025). Many of them made their money by building or scaling companies, and they carry the habits of that world into their giving. They expect evidence before they commit capital, and they increasingly want to apply the same scrutiny to philanthropy: what does a given programme actually change, and how far can a dollar be stretched before the next one does more good elsewhere?

In India, that question is remarkably difficult to answer well.

A small number of organisations abroad were built to answer exactly this kind of question. GiveWell, Founders Pledge, and Coefficient Giving have collectively directed billions of dollars by doing patient, unglamorous analysis: identifying the interventions that deliver the most good per dollar and channeling serious money toward them. Their recommendations reach hundreds of millions of people. None of them work in India.

A donor here who wants to give well, then, has relatively little to lean on. They can follow reputation, a persuasive pitch, or the choices of their peers. Independent, evidence-led advice on where money will do the most good within a specific Indian cause area is largely unavailable, because the institutions that would produce it do not yet exist here.

The scale of that omission is worth pausing on. Around 160 million people in India still live in multidimensional poverty (NITI Aayog, 2022–23). One of the fastest-growing pools of philanthropic capital in the world is forming alongside one of the largest concentrations of need in the world, and the advisory layer that would connect the two, matching capital to high-impact work, is mostly absent.

We regard that gap as unusually tractable, which is part of why we started here.

The reasoning is straightforward. Donors early in their philanthropic lives tend to be open to guidance; they have not yet committed to fixed causes or ways of working, and advice offered early tends to shape where capital eventually settles. We have seen this directly.

In our first engagement, on agricultural livelihoods, a first-generation donor came to us with a broad ambition rather than a plan: to move large numbers of people onto a better path to income. We helped turn that ambition into a fundable thesis. Working from their larger vision, we identified who was most exposed to income stress, smallholder and marginal farmers, and then, through our research, the pathway most likely to raise their incomes: diversification into higher-value agriculture and allied activities. The engagement did two things at once. It directed a confident USD 1 million commitment toward evidence-backed organisations working in specific high-need states, and it changed how the donor thought about giving itself, so that their future philanthropy is likely to start from evidence rather than instinct.

That second effect is the one we care about most, and it is the work Raising Impact exists to do. We are an evidence-led advisory for philanthropists in India. We carry out the research most donors have neither the time nor the tools to do themselves, establishing where need is greatest, where philanthropic money has the most leverage, which interventions actually work, and which organisations can credibly deliver them, and we help capital flow accordingly.

There is a legitimate question about whether the methods developed abroad transfer to India at all. This is an evidence-scarce environment, and the randomised trials that often anchor a GiveWell recommendation frequently do not exist for an Indian intervention. Our response, which the rest of this series takes up in detail, is to hold the standard of rigour fixed while broadening what we are willing to count as evidence, rather than either relaxing the bar or overstating the strength of what we have. Managing that tension between rigour and a thin evidence base is most of the job.

We are a small team, and we work deliberately in the open. We would rather set out our reasoning and invite disagreement than issue verdicts. In the pieces that follow, we describe how we work in practice: how we select a cause area, how we interrogate it, how we test our desk research against people working on the ground, and how we decide when the evidence is strong enough to move money.

The takeaway: India's philanthropic capital is growing quickly, but the infrastructure needed to deploy it well has not kept pace. Closing that gap, one well-advised commitment at a time, is why we exist

Raising Impact is an evidence-led philanthropy advisory. If you are a donor who wants your giving to be guided by evidence, we would love to talk: rukmini@raisingimpact.org

Blog 2: Intro to RIO's methodology - Longer blog

(Based on 4 part LinkedIn series + KMC post)

Title: How we pick what to fund, and what we refuse to guess

Standfirst: Everyone agrees philanthropy should fund "what works." The harder question is how you know what works. Here is our process, from cause selection to cost-effectiveness, including where we got stuck.

Blog:

Most giving starts with a cause someone already cares about. We start a step earlier, with a **question that sounds almost rude**: which problem should we even be working on?

It is an uncomfortable question, because it implies you might pass over a genuinely serious problem. But we think skipping it is exactly how well-meaning capital ends up doing less good than it could. Neglecting the prioritisation step doesn't make the trade-off disappear; it just makes it implicit. So before we recommend where a single dollar should go, we run the decision through four filters: how we choose a cause area, how we interrogate it, how we validate our desk research against reality, and how we decide what counts as sufficient evidence. **This is that process, and the moments it got hard.**

First filter: choosing where to work

We did not choose our first cause areas by asking where the need was greatest. That surprises people, so it is worth being precise about why.

Scale of a problem is necessary but not sufficient. A problem can be severe and still be a low-impact place to give, if the sector is not mature enough to absorb funding well, if no proven solutions exist yet, or if there is no credible organisation to deliver them. In the language we borrow from effective giving: **importance is only one of the variables**. Neglectedness and tractability matter just as much, and a problem that scores high on severity can still score poorly on whether additional money actually changes the outcome.

So we assessed six cause areas against four criteria: sector maturity, how much funding already flows in, how enduring the problem is, and alignment with government priority. Mental Health, Air Quality, Water and Food Systems, and One Health all made the shortlist. Two cleared it: Early Childhood Care and Education, and Income Enhancement for farmers. They scored where it

mattered, enduring problems, real funding pathways, strong policy salience, and enough sector maturity to build on.

	Priority Cause Areas	
Sector	Education & Health	Livelihoods
Theme	Early Childhood Care & Education (ECCE)	Income Enhancement
Sector Maturity Issues are well-understood, with established metrics and long-standing stakeholders driving action	Mainstream	Emerging
Existing Funding Volume and diversity of philanthropic and institutional capital	Low - Medium	High
Age of Topic How long the theme has had sectoral focus and investment	Enduring	Enduring
Prediction of Salience Priority level for the present government	Somewhat Strong	Strong

Other cause areas considered: Mental Health (Health), Air Quality (Climate), Water & Food Systems (Climate), & One Health (Climate & Health)

The genuine difficulty here was resisting the gravitational pull of severity. Every one of those six problems is important, and ranking them feels wrong. But prioritisation is not a verdict on which problem matters most in the abstract. It is a judgement about where an additional dollar shifts an outcome the most, which is a different question, and the one that actually respects the scale of the need.

Second filter: interrogating the cause area

Once we selected early childhood as our first deep dive, we needed a way to interrogate it rather than simply endorse it. We use four questions, and we call the set the NLIO lens: Need, Leverage, Interventions, Operators.

The first two are answerable from data, before speaking to anyone.

Guiding question: What are the optimal pathways for deploying philanthropic capital to maximize impact?



1 Where is the greatest need?

- How is India performing globally?
- How are states faring within India?
- Are there regional nuances?



2 Where is the highest leverage?

- How much is the Indian state spending?
- How much is spent as philanthropy?



3 What are the proven interventions?

- What are proven practices/interventions?
- What kind of evidence is available?
- What are the cost effectiveness of these interventions?



4 What and where are the reliable operators?

- Which are some credible operators?
- What is ecosystem wide infrastructure?
- What types of partnerships are required?

Need asks where the problem is worst. Here we hit our first real surprise. Maharashtra has the highest concentration of CSR funders in the country, and yet it underperforms on 11 of 17 child well-being indicators, including stunting, wasting, infant mortality, and pre-school attendance. One of India's richest states, falling short on its youngest.

Leverage asks a sharper question: where can philanthropy add value the state cannot? The numbers reset our expectations. The government spends roughly USD 1.8 billion a year on early childhood in Maharashtra. Philanthropy adds about USD 6.1 million. That is on the order of 0.3% of total spending, a ratio of more than 300 to 1.

Once you internalise that 0.3%, the counterfactual becomes obvious. Philanthropy cannot out-spend the state and should not try; at that scale, its marginal dollar is wasted on volume. Its comparative advantage is leverage: funding what the system will not, and demonstrating what the system could later scale with its own, far larger, budget. That single ratio reframed the entire cause area for us.

Need and Leverage tell you where to work and why. They do not yet tell you what works, or who can deliver it. For that, we had to leave the spreadsheet.

Third filter: checking the data against reality

Our secondary analysis produced a result that looked, at first, like an error. It flagged Thane, Pune, and Kolhapur, three of Maharashtra's most prosperous districts, with strong public and

philanthropic spending, as underperforming on child health. On paper it made no sense. Prosperous districts should not be failing their children.

We could have discarded the finding as noise. Instead we did what we now do with every desk finding before we trust it: we tested it against people who work in these districts. Over the course of this cause area, we spoke with practitioners and researchers whose day-to-day work is exactly this, among them a senior leader at one of India's largest education non-profits, a physician who has published on immunisation coverage in Pune, a former Director of Health Services for the Government of Maharashtra, a community-medicine researcher who studied Anganwadi attendance in Mumbai, and a former director of a national population-sciences institute.

They confirmed the finding, and then explained what the numbers were concealing. Prosperous districts contain large urban-slum populations and fast-growing peri-urban outskirts, and those pockets are frequently the least served in the district. The district average is high precisely because the surrounding wealth is real, and the average buries the households who need the most. The prosperity and the neglect sit side by side; from a distance, only the average is visible.

This is the entire reason we do not stop at secondary data. Quantitative analysis tells you where to look. Domain experts tell you whether you are reading it correctly, and surface the mechanisms a dataset cannot, how a problem actually behaves on the ground. Those same conversations do a second job: they help separate interventions that work from interventions that merely sound good.

Early childhood interventions fall into four broad types, awareness, such as counselling on feeding and newborn care; capacity building, strengthening frontline-worker skills; systems strengthening, improving how existing services are delivered; and direct beneficiary transfer, resources given straight to families.

And here is the finding that should reshape how the sector allocates. Roughly 60% of philanthropic funding for early childhood in India goes to infrastructure and direct delivery. Yet the available evidence points to awareness and systems interventions as substantially more cost-effective per outcome, in several cases by close to an order of magnitude. The most-funded category is rarely the most cost-effective one. ***(We build a full cost-effectiveness model for each intervention type, comparing cost per outcome across them; that analysis is involved enough to deserve its own blog.)***

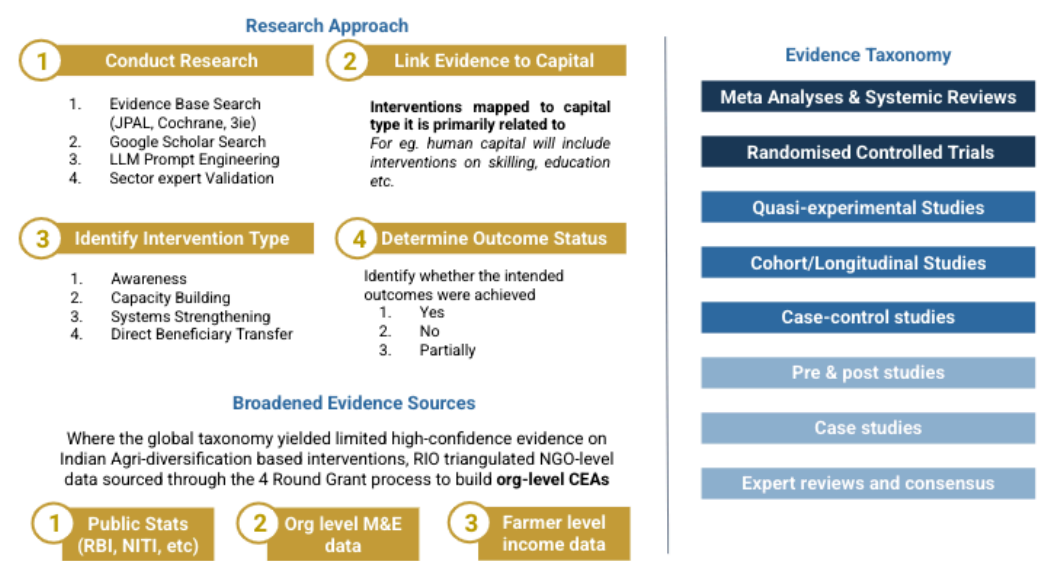
That leaves the final question in the lens: who can actually deliver? A proven intervention is only as good as the organisation implementing it, so we map credible operators, the ones with

the reach, the fidelity, and the track record to make it real. Only with all four answers, Need, Leverage, Interventions, Operators, do we have a defensible picture of where philanthropy can move the needle.

Fourth filter: holding the evidence bar

A promising intervention delivered by a capable operator still is not a decision. The evidence itself has to clear a bar. And this is where working in India gets genuinely hard, because the strongest evidence for what works here is often not Indian.

We rank evidence by how much weight it can bear: meta-analyses and systematic reviews at the top, then randomised controlled trials, then quasi-experimental studies, cohort studies, pre-post studies, and expert consensus at the base. The higher up the ladder, the more confidently we can act on a claim.

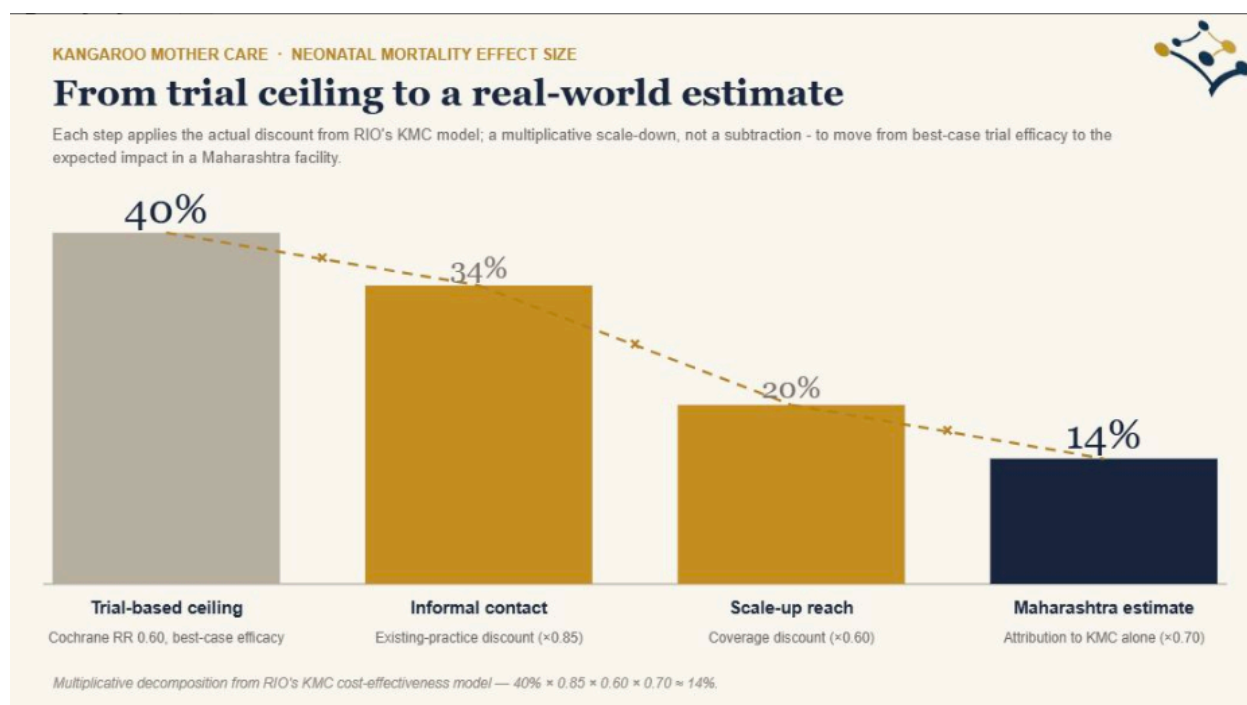


The problem is that for many interventions in India, the top of the ladder is empty. The rigorous trials were run elsewhere. We refuse to treat that as a dead end, and we equally refuse to lower the bar. Instead we broaden the base, triangulating strong global evidence with public data from bodies like NITI Aayog and RBI, and with monitoring data from implementing organisations.

The discipline is holding two ideas at once: take the best available evidence seriously wherever it was produced, and stay honest that an effect measured in one context rarely transfers cleanly to another. That second half is the unglamorous core of the work, and it is where most cost-effectiveness estimates quietly go wrong.

Here is what it looks like in practice. We were building the case for Kangaroo Mother Care, skin-to-skin contact for underweight newborns, one of the most evidence-backed, low-cost interventions in newborn survival. We wanted a single figure for how much it reduces a newborn's risk of dying, grounded in evidence applicable to Maharashtra. The number quoted almost everywhere is a 40% reduction in mortality. It is a real result, but it is pooled from a subset of global trials across very different health systems, and in several of them it is unclear whether the comparison arm received the same breastfeeding support as the treatment arm. In effect, 40% is a best case from elsewhere, and it almost certainly overstates what a real Maharashtra ward, at real-world compliance, would achieve.

So we did not adopt 40% as our estimate. We treated it as a ceiling and applied three discount factors: for the skin-to-skin contact that already occurs informally in Indian wards (which shrinks the counterfactual gain), for the share of newborns who would actually receive the intervention once a programme scales past a trial, and for the fraction of the mortality benefit attributable to KMC itself rather than the bundle of care it usually accompanies. Compounded, those adjustments brought our working estimate from 40% down to roughly 14%, a little over a third of the headline figure.



That is not pessimism; it is calibration. A global effect size is a prior, not an answer, and the real work is adjusting it for your context one assumption at a time.

What does this add up to?

Multiple filters, and only then a recommendation. Choose the cause area more than severity. Interrogate it on need and leverage. Validate the desk work against people who know the

ground. And hold the evidence to a bar we will not relax, even when the cleanest study was run on another continent.

None of this is fast, and very little of it is glamorous. But it is the difference between giving that sounds good and giving you can defend.

The takeaway: Rigour in an evidence-scarce context is not about waiting for perfect evidence. It is about being disciplined with imperfect evidence, explicit about uncertainty, and willing to leave the spreadsheet to reduce it. That discipline is the product.

This is how we work. If you are a donor who wants your giving held to this standard, we would like to hear from you: rukmini@raisingimpact.org